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Government and Medicine: The Two Horsemen of a Global Pandemic

For many, Covid began during the spring of 2020. Yet for the more attentive ones, the people who kept a finger on the pulse of the world, reports of a deadly virus originating in China had been circulating the globe since December. I remember hearing about it heavily in January, but being the only person in my grade who voluntarily listened to NPR on the way to school, my concerns were not held by my classmates. I distinctly recall the beach vacation my family and friends took together where we received the news that school would be delayed for two weeks. Everyone remembers those two weeks; they were the beginning of the end, a naive embrace to this new “flu.” The following months were full of regulation, bizarre global politics, rabid pushback to masks and government mandates, a medical consensus as clear as mud, and enough toilet paper shortages to make the Charmin toilet paper bears run and hide. While many of the experiences we had during the COVID-19 pandemic mirrored those of the global population during the 1918 pandemic, lack of medical equipment and increased geopolitical stresses made the perfect substrate for the 1918 flu to wreak havoc across the world, leaving a population scarred and crippled by the deadly virus.

To begin, according to an article by The New York Times, the 1918 influenza outbreak killed over three million people worldwide.¹ This, as you may imagine, is a

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7. Valeria Gawron, "In Memorial: Joseph and Stanley Garas," Pandemic Influenza Storybook: Stories, CDC, https://archive.cdc.gov/#/details?url=https://www.cdc.gov/publications/panflu/stories/memorial_garas.html
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gross misreporting, as actual figures place the death count at about 50 million.² With a total infection count of 500 million and a total population of just under 2 billion, that leaves the infection rate and death rate of 25% and 2.5%, respectively. Compare this to the 3.9% infected and 0.043% dead by the COVID-19 virus³ and it really begins to put into perspective just how viciously the 1918 flu ripped through the world's population. I believe that the primary reason for this drop in infections and deaths is the obvious advance in medical equipment. The use of ventilators, N-95 masks, and a much more robust healthcare system led to a worldwide population that was much more prepared to handle the pandemic. In Camp Devens, a doctor named Roy states in his letter to his friend Burt that "it is only a matter of a few hours then until death comes, and it is simply a struggle for air until they suffocate. It is horrible. One can stand it to see one, two or twenty men die, but to see these poor devils dropping like flies sort of gets on your nerves. We have been averaging about 100 deaths per day, and still keeping it up."⁴ This struggle of survival makes sense when you hear the instructions the doctors had to keep their patients alive: "Treatment under direction of the physician is simple, but important, consisting principally of rest in bed, fresh air, abundant food, with Dover's powder for relief of pain."⁵ The only real medicine in that plan is Dover's powder, though the efficacy of opium powder for a virus is questionable. Through both personal experience and these sources, it is clear that for COVID-19, humans were well equipped to battle the virus, armed to the teeth with fast vaccinations, DNA-sequencing, ultrafine particle filters, and extreme cleaning products. On the other hand, the plan for recovery in 1918 was little more than "sit back and pray," allowing the flu to take every victim it could.

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The nurses and physicians are not the only ones to blame for the spread of the virus. Looking into the sources of the time, it is clear that the governments of the world had a much larger impact on the spread of the flu than many would like to believe. Through readings of letters sent in and out of Camp Funston, the vagueness and lack of importance in New York Times articles, and worldwide reporting of the flu, many would conclude that information about the flu was a closely guarded secret. In a letter to Camp Funston by Governor Arthur Capper, the governor asks the general to “wire or write me as soon as possible a statement of actual conditions especially to the care that is being given sick soldiers.” This message comes after a public outcry at the obscurity of the care given to the soldiers. To understand why US soldiers were so heavily affected by the flu, one must realize that the majority of the young men in the US were involved in war efforts for WWI. Due to the US’s need to present a strong force to its allies, it was business as normal for many, which led to severe outbreaks throughout military camps where soldiers were housed in cramped barracks. During the COVID-19 pandemic, countries were quick to openly communicate the status of their population, updating infection and death numbers daily during the height of the virus. Instant communication around the world meant doctors were up-to date on the latest techniques and effective protocols, in addition to the population adopting social distancing, work from home, home quarantine, and mask etiquette much more quickly and widespread than that in 1918. An article posted in the edge of the New York Times put it best, “When the history of this influenza epidemic comes to be written, it will not reflect much glory on medical science... It will have to be admitted that though the members of the United States had long and full warning that the infection would come to this country, when it arrived they

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did little more than tell us that among a well-fed people there would be no widespread infection.”⁶ It goes on to explain that “That makes strange reading, now, with the deaths more numerous in our training camps than anywhere else. One recalls, too, that to early suggestions of strict quarantine the reply was made that such measures not be taken because they would interfere with the movements of our troops and the general conduct of our war business.”⁶ Looking into history’s pages, it is clear that the US government had a large and invisible hand in the rampant spread of the flu.

The final aspect of both the COVID-flu and the 1918 epidemic is more invisible. In many reports, numbers focus on the dead, highlighting the effect the flu has on those no longer living. This makes sense, as naturally we pay close attention to factors that affect our survival. Yet something not mentioned often is the mental and physical effect that something like a worldwide pandemic has on those who survive. For those of us still living in the wake of the COVID-19 pandemic, the effect is obvious: an entire generation of young students and children forced to live a significant portion of their lives locked away, unable to connect with others. For many of my classmates, it stunted their academic progress. For me, it forced my senior year to be one of social isolation, meaningless take home exams, and non-existent sport’s seasons. For countless people, it meant the death of immediate family, the vaporization of savings, and the fragility of their economic situation as shops were shuttered, people were fired, and the world went into survival mode. The 1918 pandemic was no different, with many first hand accounts talking briefly about the mental toll it had on the survivors. In a memoir, Paul Kendall talks about how his grandfather talked about his experience collecting bodies from the local town, bringing them back to field hospitals. The gruesome task

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was given to many young men studying religion, but had clear and harsh repercussions. Paul, in talking about his grandfather, says “It deeply affected him to have seen so much death at such a young age.”⁷ Another memoir by Valeria Gawron talks about her grandfather’s experience with the pandemic, highlighting what I believe many of us today feel. Through her words, her grandfather said that the epidemic “is a thief who steals the young and the old and gives no time to mourn.”⁸ In addition to this, when talking about the graves of her uncles, Valeria’s cousin asked why there were so few gravestones in the area of the cemetery, when there were clearly people buried there. Her grandfather replied with a chilling remark, telling her “We wanted to forget.”⁸ I believe that is the sentiment many of us have today. Even during the pandemic, many of us were looking forward to when masks were no longer needed for every interaction, when we could go back to hugging people and social gatherings, where schools met and church was held without fear of being part of a “super spreader event.” Today, many of us forget the harsh reality of the world closing down, the grimness of the pandemic resurging only when we find a spare cloth mask tucked behind a car seat or into the pockets of a winter coat seldom worn.

The tales of the 1918 pandemic have been abandoned to history books, yet the effects are still with us today, infused in the culture around the world. Personally, reading letters, news articles, and memoirs about the 1918 flu was like being thrown back into the middle of the pandemic, a blend of confusion, fear, and utter helplessness as we watched the world around us suffer at the hands of the virus. The secretive, taboo nature of the 1918 flu mirrors the sentiment many have towards COVID-19 today, as many choose to ignore the devastating impact it had on the world’s population,

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economy, and mental health. Despite better medical equipment and more organized governments, both the 1918 flu and the COVID-19 pandemic had immense and deep-rooted effects on our daily life. In conclusion, the reports from the early 20th century mirror my personal experiences since 2020, and digesting these reports gave me a much better understanding about how the world handles worldwide pandemics, both back then and now.

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